

Locking Away Prosperity? Evaluating the Economic Impact of the Key to NYC Program

0.1 Introduction

The COVID-19 pandemic triggered a global wave of non-pharmaceutical interventions (NPIs) as governments acted swiftly to limit viral transmission. Much early research focused on the public health rationale for these policies—especially the importance of early action to contain exponential spread. For instance, [Friedson et al. \[2021\]](#) and [Yang \[2021\]](#) use synthetic control methods to evaluate lockdowns in California and Wuhan, emphasizing the critical role of timing. Similarly, [Bayat et al. \[2020\]](#) estimate that New York City could have reduced COVID-19 deaths by over 80% had its first lockdown occurred just ten days earlier. Subsequent work examined the economic consequences of such interventions. While NPIs helped flatten the curve, they also raised concerns about business closures, layoffs, and lasting economic scarring. Studies like [Gibson and Sun \[2023\]](#) and [Lee and Yang \[2022\]](#) document labor market costs of lockdowns, while [Ke and Hsiao \[2022\]](#) show that although strict NPIs incur short-term economic losses, those effects may be temporary.

By 2021, as vaccines became widely available, cities turned to vaccine mandates as a less restrictive (but still controversial) tool for curbing transmission. Unlike lockdowns, mandates aimed to sustain economic activity while reducing health risks in high-contact settings. Nevertheless, critics argued that requiring proof of vaccination for indoor dining and entertainment could slow employment recovery in restaurants by alienating customers or exacerbating staffing shortages. Supporters countered that mandates would improve public safety and restore consumer confidence, potentially boosting demand and employment. These competing narratives played out most visibly in four large U.S. cities that adopted vaccine mandates for indoor leisure activities: New York City, San Francisco, New Orleans, and Los Angeles.

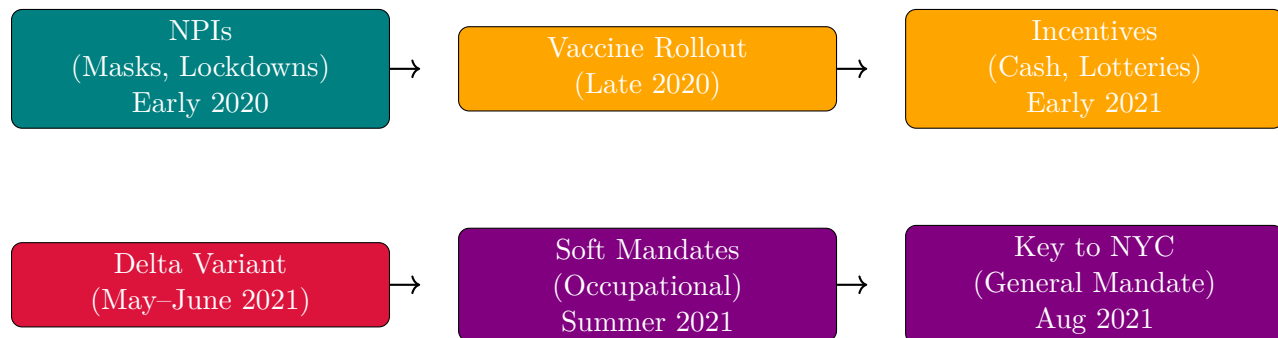
Each city’s mandate differed in timing and design. New York City’s *Key to NYC* program, announced in August 2021 and enforced beginning September, was the first large-scale vaccine mandate for indoor commerce in the United States. San Francisco followed with an even stricter rule requiring full vaccination for customers and employees, effective later that same month. New Orleans adopted a hybrid approach, permitting either proof of vaccination or a recent negative test, while Los Angeles—by far the largest jurisdiction—implemented its policy in November. These mandates created a small but consequential set of urban outliers in the national policy landscape, with important implications for labor markets in the leisure

and hospitality sector.

This paper evaluates the causal effects of these city-level vaccine mandates on restaurant employment, with particular emphasis on New York City’s *Key to NYC* program. While prior research has analyzed mandates’ impacts on vaccination rates and health outcomes [Cohn et al., 2022, Acharya and Dhakal, 2021, Lang et al., 2023], relatively little is known about their labor market consequences, especially in industries dependent on face-to-face interaction. This study helps fill that gap. Because randomized experiments are infeasible, the analysis adopts a synthetic control approach to estimate the counterfactual trajectory of restaurant employment absent the mandates. Using MSA-level monthly data from the Federal Reserve Bank of St. Louis, I construct synthetic controls for each mandate city’s year-over-year growth in full-service restaurant employment. The donor pool consists of metropolitan areas that did not implement similar mandates during the same period.¹

The rest of the paper is organized as follows. Section 1 provides background on the evolution of pandemic interventions and the city-level vaccine mandates. Section 2 describes the employment data and outcome construction. Section 3 outlines the empirical strategy, including the synthetic control methodology. Section 4 presents the main findings and robustness checks. Section 5 interprets the results and discusses limitations. Section 6 concludes.

1 Policy Background



Timeline of major pandemic policy phases

1.1 NPIs Before Vaccines

NPIs had aimed to reduce transmission by limiting contact or promoting respiratory barriers. The first and most famous of these interventions was the 2020 lockdown in Wuhan, China, where local officials went as far as to forcibly seal people in their homes to reduce the possibility to spread the virus; similar actions were taken in Europe and India. Even in the United States, states that could effectively seal their borders, such as Hawaii, did so, with Hawaii mandating two-week quarantines to visitors, which effectively amounted to a

¹For example, ‘SMU11000007072251101SA’ (Washington, DC) or ‘SMU08197407072251101SA’ (Denver MSA).

tourism ban. Mask mandates were widely adopted as well due to their low cost and direct mechanism of action [Hansen and Mano, 2023, Chernozhukov et al., 2021, Mitze et al., 2020]. While not as severe as lockdowns, the idea of mask mandates was to allow for some freedom of movement while minimizing the chance to spread the virus further. While both sets of policies were effective, they provoked political and economic backlash due to their disruptive nature.

1.2 The First Vaccine Policies

The arrival of vaccines in late 2020 ushered in a new phase of pandemic governance, giving jurisdictions a wider array of policy options to employ. Now, instead of lockdowns, governments finally had pharmaceutical interventions in their toolkit. For the first few months, it worked: when vaccines were released, initial uptake was high, particularly among older adults and (often mandated for) healthcare workers. However, as eligibility expanded in early 2021, demand slowed amid misinformation, mistrust, and logistical barriers. In response, policymakers deployed incentive-based strategies to encourage vaccination [Barber and West, 2022, Robertson et al., 2021]. These included direct cash payments, vaccine lotteries (e.g., Ohio’s “Vax-a-Million” from May to June [Fuller et al., 2022]), and smaller symbolic giveaways. One paper dubbed this the era of “donuts, drugs, booze, and guns,” reflecting the extraordinary lengths to which governments went to boost uptake through rewards rather than requirements [Tinari and Riva, 2021].

Despite their creativity, most incentives had modest or short-lived effects [Saban et al., 2021, Walkey et al., 2021, Khazanov et al., 2023]. Many were poorly targeted, administratively complex, or insufficient to overcome deeper resistance. The emergence of the highly transmissible Delta variant in mid-2021 raised the stakes. Initially identified in India, Delta reached the United States by May and accounted for the majority of new cases by June. Unlike in 2020, officials now had vaccines at scale—but not enough vaccinated people. Given a new more contagious strain, soft mandates emerged, targeting workers in high-risk occupations such as healthcare and education. These policies were not general mandates but did make vaccination a condition of continued employment [American Medical Association et al., 2021]. Over the summer, local governments and organizations expanded this approach [American Medical Association et al., 2021, Al Jazeera, 2021].

1.3 City-Level Vaccine Mandates

The summer 2021 Delta surge spurred several major U.S. cities to enact vaccine verification requirements for indoor restaurants and leisure venues. These policies were concentrated in just four jurisdictions: New York City, San Francisco, New Orleans, and Los Angeles. Each city’s approach differed slightly in timing and scope, but all shared the common goal of linking access to indoor leisure activities with proof of vaccination. Together, they formed the clearest urban experiments in the use of vaccine mandates to manage both public health risks and consumer confidence in the hospitality sector.

New York City’s “Key to NYC” program was the first such mandate in the United States. Announced on August 3, 2021, formalized on August 16, and enforced beginning September

13, it required proof of at least one vaccine dose to enter indoor dining, fitness centers, and entertainment venues. Importantly, the rule applied not only to patrons but also to employees of covered establishments. This dual requirement meant that the policy had the potential to affect restaurant employment through both the supply side, if workers resisted vaccination or exited the industry, and the demand side, if consumer traffic shifted in response to the mandate. The city threatened fines for noncompliance and required businesses to post signage and maintain written protocols, though the intensity of inspections varied. Large chains tended to comply quickly, while smaller establishments voiced concern about administrative burdens and customer pushback. Several lawsuits challenged the mandate, but city officials argued that rising vaccination rates demonstrated its effectiveness.

San Francisco followed closely, enacting its mandate on August 20, 2021. Unlike New York, the city required full vaccination rather than just one dose, and did not allow a testing alternative. This made San Francisco's policy the strictest among the four cities. The requirement extended to both customers and employees in indoor restaurants, bars, and entertainment venues. Enforcement included spot checks and the threat of fines, with compliance reported to be widespread among larger businesses in the city's dense urban core. As in New York, officials promoted the policy as a tool for stabilizing consumer confidence in public spaces, even as smaller businesses expressed anxiety about turning customers away.

New Orleans became the first major city in the South to adopt a mandate, announcing its policy on August 16, 2021. The city's approach was a hybrid model: individuals could gain entry to indoor restaurants and venues either by showing proof of at least one vaccine dose or by presenting a recent negative COVID test. This flexibility reflected local political and cultural dynamics but also introduced more administrative complexity for restaurants. Enforcement began within weeks, with inspections carried out by city officials. Restaurant owners reported mixed reactions, with some noting relief that the policy might make indoor dining safer, while others expressed concern about deterring unvaccinated tourists and residents.

Los Angeles entered the field later, passing its ordinance in October 2021 with enforcement beginning November 4. The policy was sweeping in scope, covering indoor restaurants, gyms, theaters, salons, and other leisure establishments. By the time it came into effect, public debate had already been shaped by the experience of New York and San Francisco, and Los Angeles became the largest jurisdiction in the nation to implement a vaccine passport. Enforcement mechanisms were formally in place, though reports suggested that actual inspections were inconsistent. Many smaller establishments adopted a pragmatic approach, balancing legal requirements with customer management.

Although the four cities differed in timing and strictness, they shared important common features. Each policy was applied citywide, targeted indoor leisure and hospitality settings, and created new compliance responsibilities for businesses. They also diverged sharply from the policy environments of states such as Texas and Florida, which prohibited vaccine requirements altogether. The result was a small cluster of urban outliers using vaccine mandates as a central public health intervention in 2021.

These institutional details carry important implications for research design. Because the

mandates covered both patrons and employees in most cases, they may have constrained labor supply by excluding some workers while simultaneously influencing labor demand by shaping customer traffic. Enforcement intensity and public perception varied, but each policy represented a reasonably sharp and well-documented intervention, announced and enforced on a specific timeline. This combination of clarity and heterogeneity makes them suitable for empirical evaluation. Comparing employment outcomes in these cities against synthetic control groups of metropolitan areas without mandates offers a credible way to estimate causal effects on restaurant employment. The staggered adoption across cities and the variation in stringency further enrich the research design, providing multiple natural experiments within a single policy family.

1.4 The First Vaccine Mandates

Per the above, vaccine mandates could influence employment in full-service restaurants through both supply- and demand-side channels, with theoretically ambiguous net effects. On the supply side, some workers may exit the labor force or shift sectors if unwilling to comply, especially in lower-wage service jobs where vaccine hesitancy may be higher. However, others may comply with the mandate precisely *because* they wish to keep their jobs, choosing continued employment over personal objection. That is, the employment effect may hinge less on individual preferences in isolation and more on the strength of the economic incentives at stake. Employers, meanwhile, may face frictional costs from enforcing mandates or accommodating staffing turnover. On the demand side, mandates may increase consumer confidence by signaling a safer environment, thereby encouraging more patrons to return to indoor dining. For a sector like full-service restaurants, where both interpersonal contact and discretionary spending are central, mandates could plausibly stimulate demand even as they introduce short-run supply constraints. Whether employment rises or falls in net depends on which margin dominates. In short, this analysis seeks to quantify the net effect of the mandate on restaurant employment, capturing the balance between these potentially opposing forces.

2 Dataset

We use monthly MSA-level employment series from the Federal Reserve Bank of St. Louis (FRED) and, by extension, the Bureau of Labor Statistics, spanning January 1991 through December 2022, yielding $T = 384$ monthly observations per unit. The pre-treatment period consists of the set $\mathcal{T}_1 = \{t \in \mathbb{N} : t \leq T_0\}$, where T_0 is the final period before treatment. Similarly, the post-treatment period is given by $\mathcal{T}_2 = \{t \in \mathbb{N} : t > T_0\}$.

Our analysis focuses on two related outcome sets: (1) full-service restaurant employment for 30 MSAs and New York City (FRED Code: SMU36935617072251101SA) and (2) broader leisure industry employment at the MSA level for approximately 255 MSAs from 1991 to December 2022, with New York City (FRED code SMU36935617072251101SA) as the treated unit.

Let e_{jt} denote employment in unit j at month t . For both the full-service restaurant analysis

and broader leisure market analysis, the outcome is constructed as the year-over-year growth rate in percentage points,

$$y_{jt} = 100 \cdot \frac{e_{jt} - e_{j,t-12}}{e_{j,t-12}}, \quad j = 1, \dots, N, \quad t = 13, \dots, T.$$

of the smoothed and seasonally adjusted values. Outcomes are collected into vectors $\mathbf{y}_j = (y_{j1}, \dots, y_{jT})^\top$, and the donor pool is defined as $\mathcal{N}_0 = \mathcal{N} \setminus \{1\}$ with $N_0 = 29$ MSAs as donors for the full-service restaurant industry. The reason there are 29 donors instead of more is because BLS only takes *full service restaurant* data for only a few MSAs. Beyond this, we focus on full-service restaurants due to their direct exposure to the mandate, which required vaccination proof for indoor dining, unlike limited-service establishments where takeout predominates.

The Key to NYC policy, implemented in August 2021, designates July 2021 as the last pre-treatment month T_0 , with the treatment indicator defined by

$$d_{jt} = \mathbf{1}\{j = 1 \wedge t > T_0\}.$$

Observed outcomes satisfy

$$y_{jt} = (1 - d_{jt})y_{jt}(0) + d_{jt}y_{jt}(1),$$

with causal effects $\tau_{jt} = y_{jt}(1) - y_{jt}(0)$ and post-treatment average treatment effect on the treated (ATT)

$$\text{ATT} = \frac{1}{|\mathcal{T}_2|} \sum_{t \in \mathcal{T}_2} \tau_{1t},$$

where $\mathcal{T}_2 = \{t > T_0\}$.

3 Econometric Methodology

Unfortunately, Standard SCM yielded inadequate pre-treatment fit. In order to generate the counterfactual, I needed to allow extrapolation beyond the convex hull. To estimate the counterfactual employment trajectory for NYC's full-service restaurant sector and the broader employment trends in the leisure labor market, I therefore employ a synthetic control approach that hybridizes Forward Selection SCM (FSCM) and the Augmented SCM (ASCM) estimators. The classical Synthetic Control Method (SCM) solves

$$\mathbf{w}^{\text{SCM}} = \underset{\mathbf{w} \in \mathcal{W}_{\text{conv}}}{\text{argmin}} \left\| \mathbf{y}_1^{\mathcal{T}_1} - \mathbf{Y}_0^{\mathcal{T}_1} \mathbf{w} \right\|_2^2,$$

where

$$\mathcal{W}_{\text{conv}} = \left\{ \mathbf{w} \in \mathbb{R}_{\geq 0}^{N_0} \mid \mathbf{1}^\top \mathbf{w} = 1 \right\}$$

is the probability simplex. This finds the weights $\mathbf{w}^{\text{SCM}}$ that best reconstruct the treated unit's pre-treatment outcomes as a convex combination of donor outcomes.

3.1 Forward Selection SCM

FSCM constructs the synthetic control iteratively by adding donor units one at a time. Starting with an empty selected donor set $\mathcal{S} = \emptyset$, at each iteration the algorithm evaluates each candidate donor $j \in \mathcal{N}_0 \setminus \mathcal{S}$ by forming an expanded set $\mathcal{S}' = \mathcal{S} \cup \{j\}$. For each $\mathcal{S}'$, it solves the restricted SCM problem over the donor submatrix $\mathbf{Y}_0^{\mathcal{S}'}$, consisting of columns corresponding to units in $\mathcal{S}'$:

$$\mathbf{w}_{\mathcal{S}'}^* = \underset{\mathbf{w} \in \mathcal{W}_{\text{conv}}(\mathcal{S}')}{\operatorname{argmin}} \left\| \mathbf{y}_1^{\mathcal{T}_1} - \mathbf{Y}_0^{\mathcal{T}_1, \mathcal{S}'} \mathbf{w} \right\|_2^2,$$

where

$$\mathcal{W}_{\text{conv}}(\mathcal{S}') = \left\{ \mathbf{w} \in \mathbb{R}_{\geq 0}^{|\mathcal{S}'|} \mid \mathbf{1}^\top \mathbf{w} = 1 \right\}.$$

The donor selected at iteration k is

$$j^* = \underset{j \in \mathcal{N}_0 \setminus \mathcal{S}}{\operatorname{argmin}} \min_{\mathbf{w} \in \mathcal{W}_{\text{conv}}(\mathcal{S} \cup \{j\})} \left\| \mathbf{y}_1^{\mathcal{T}_1} - \mathbf{Y}_0^{\mathcal{T}_1, \mathcal{S} \cup \{j\}} \mathbf{w} \right\|_2^2,$$

and $\mathcal{S}$ is updated as

$$\mathcal{S} \leftarrow \mathcal{S} \cup \{j^*\}.$$

This forward selection continues until a stopping criterion is met, such as reaching a maximum number of donors, a minimum improvement threshold, or an information criterion like modified BIC.

3.2 The Augmented Synthetic Control Estimator

Building on the baseline SCM, ASCM relaxes the convex weight restriction by allowing affine weights and adds a regularization penalty that shrinks the solution towards a reference weight vector $\mathbf{w}_0$, typically the SCM or FSCM solution. The augmented objective is

$$\mathbf{w}^{\text{aug}} = \underset{\mathbf{w} \in \mathcal{W}_{\text{aff}}}{\operatorname{argmin}} \left\| \mathbf{y}_1^{\mathcal{T}_1} - \mathbf{Y}_0^{\mathcal{T}_1} \mathbf{w} \right\|_2^2 + \lambda \left\| \mathbf{w} - \mathbf{w}_0 \right\|_2^2,$$

where $\lambda \geq 0$ controls the penalty strength and

$$\mathcal{W}_{\text{aff}} = \left\{ \mathbf{w} \in \mathbb{R}^{N_0} \mid \mathbf{1}^\top \mathbf{w} = 1 \right\}$$

is the affine subspace of weights summing to one, but not restricted to be non-negative. The intuition is that when $\mathbf{w}_0$ (e.g., FSCM weights) provides a good fit, λ will be large to maintain interpretability; when fit needs improvement, λ can be small to allow extrapolation beyond the convex hull.

3.3 Forward Augmented Synthetic Control

FASC integrates the interpretability of FSCM with the flexibility of ASCM by first performing forward selection to obtain $\mathbf{w}^{\text{FSCM}}$, then solving

$$\mathbf{w}^{\text{FASC}} = \underset{\mathbf{w} \in \mathcal{W}_{\text{aff}}}{\operatorname{argmin}} \left\| \mathbf{y}_1^{\mathcal{T}_1} - \mathbf{Y}_0^{\mathcal{T}_1} \mathbf{w} \right\|_2^2 + \lambda \left\| \mathbf{w} - \mathbf{w}^{\text{FSCM}} \right\|_2^2,$$

where $\lambda \geq 0$ balances fidelity to the sparse forward-selected weights and pre-treatment fit.

Large λ yields a solution close to $\mathbf{w}^{\text{FSCM}}$, preserving sparsity and interpretability, whereas small λ prioritizes fit, allowing positive or negative weights outside the original selected donors.

3.3.1 Cross-Validation for λ

The hyperparameter λ is chosen by time-split cross-validation over the pre-treatment period. The training set (first half of pre-treatment) is used to estimate

$$\mathbf{w}^{\text{FASC}}(\lambda) = \underset{\mathbf{w} \in \mathcal{W}_{\text{aff}}}{\operatorname{argmin}} \left\| \mathbf{y}_{\text{train}} - \mathbf{Y}_{0,\text{train}} \mathbf{w} \right\|_2^2 + \lambda \left\| \mathbf{w} - \mathbf{w}^{\text{FSCM}} \right\|_2^2,$$

while the validation set (second half of pre-treatment) evaluates out-of-sample error via

$$\text{CV-RMSE}(\lambda) = \left\| \mathbf{y}_{\text{val}} - \mathbf{Y}_{0,\text{val}} \mathbf{w}^{\text{FASC}}(\lambda) \right\|_2.$$

The λ minimizing CV-RMSE balances interpretability and fit. Bayesian optimization over $\log_{10}(\lambda) \in [-2, 3]$ with Gaussian Process surrogates and Expected Improvement acquisition efficiently explores this space. Since $\mathbf{w}^{\text{FSCM}}$ is feasible in the FASC optimization, the in-sample Mean Squared Error (MSE) satisfies

$$\text{MSE}^{\text{FASC}} = \frac{1}{T_0} \left\| \mathbf{y}_1^{\mathcal{T}_1} - \mathbf{Y}_0^{\mathcal{T}_1} \mathbf{w}^{\text{FASC}} \right\|_2^2 \leq \frac{1}{T_0} \left\| \mathbf{y}_1^{\mathcal{T}_1} - \mathbf{Y}_0^{\mathcal{T}_1} \mathbf{w}^{\text{FSCM}} \right\|_2^2 = \text{MSE}^{\text{FSCM}},$$

ensuring FASC's fit is never worse and typically better than FSCM.

4 Results

5 Discussion

6 Conclusion

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